

STROKE UNIT DELIRIUM SCREENING FORM

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Companion to Stroke Unit Delirium Screening Protocol — Use with 4AT, CAM, ICDSC, or CAM-ICU per protocol Section 5.2

Patient Name:	MRN:	DOB:
Admission Date/Time:	Stroke Type (Ischemic / ICH / SAH / TIA):	Admission NIHSS:
Unit / Bed:	Attending / Stroke Team:	Post-tPA/Thrombectomy? Y / N — Time:

1. Reference Neurological / Cognitive Exam (Complete Once on Admission; Update if Deficits Evolve)

Per protocol Section 5.1 — this establishes the stroke-related baseline against which every subsequent delirium screen must be interpreted. A screen is only truly positive when it reflects a change superimposed on this baseline.

Domain	Findings
Pre-stroke baseline mentation (patient/family report)	
Aphasia (type: expressive / receptive / global / none)	
Neglect (side, if present)	
Hemianopia / visual field deficit	
Dysarthria	
Baseline level of consciousness / RASS at rest	
Other relevant deficits	
Reference exam completed by (name/credential, date/time)	

2. Screening Tool Quick Reference

2.1 4AT (Rapid Screen — use with caution if known aphasia/neglect; confirm positives with CAM)

Item	Scoring	Score
Alertness	Normal = 0; Mild sleepiness >10 sec or agitation = 4	☐ 0 ☐ 4
AMT4 (age, DOB, place, current year)	No mistakes = 0; 1 mistake = 1; ≥2 mistakes = 2	☐ 0 ☐ 1 ☐ 2
Attention (months backward from December)	≥7 months correctly = 0; starts but <7 / refuses = 1; untestable = 2	☐ 0 ☐ 1 ☐ 2
Acute change or fluctuating course	No = 0; Yes = 4	☐ 0 ☐ 4

4AT Total Score: _____ (0 = delirium unlikely; 1–3 = possible cognitive impairment; ≥4 = possible delirium — escalate per Section 4 below)

2.2 CAM (Preferred in Stroke — Best Validated Against DSM Criteria)

Positive CAM = Features 1 AND 2, plus either Feature 3 OR Feature 4:

	Feature	Present? (interpret against reference exam)
☐	1. Acute onset or fluctuating course	☐ Yes ☐ No
☐	2. Inattention	☐ Yes ☐ No
☐	3. Disorganized thinking	☐ Yes ☐ No
☐	4. Altered level of consciousness	☐ Yes ☐ No

CAM Result: ☐ Positive (1+2, and 3 or 4) ☐ Negative

3. Shift-by-Shift Screening Log

Complete at each required interval per protocol Section 5.3 (every 12 hours if high-risk / neuro-ICU / first 48h post-thrombectomy; every 24 hours if standard risk; and immediately with any acute change).

Date / Time	Shift	Tool Used	RASS	Score	Positive?	Attributable to baseline deficit? (Y/N)	RN Initials	Provider Notified (time) / Action

4. Positive Screen — Immediate Actions (Protocol Sections 6–7)

Do Not Assume Delirium First

For any positive screen or acute change, notify the stroke team within the shift and complete the differential work-up before managing as delirium alone: STAT non-contrast head CT if new/worsening neuro deficit is suspected; consider STAT EEG if unexplained fluctuating unresponsiveness (rule out non-convulsive seizure); check for infection (aspiration pneumonia, UTI), metabolic derangement (glucose, sodium — especially with SAH), hypoxia, and review medications for deliriogenic or newly added agents.

Action	Completed (initials / time)
Notified bedside RN / charge nurse	
Notified stroke team (resident / APP / attending)	
STAT head CT ordered if new/worsening deficit suspected	
EEG considered if fluctuating unresponsiveness / seizure suspicion	
Infection work-up initiated if indicated	
Metabolic panel / glucose / sodium checked	
Medication review completed (pharmacy notified if needed)	
Non-pharmacologic prevention bundle initiated (Section 8 of protocol)	
Family notified / engaged at bedside	

5. Resolution

☐ Two consecutive screens negative or stable relative to reference exam ☐ date/time resolved: _____	
RN signature: _____ Provider co-signature (if applicable): _____	